

Criteria	Yes	No
Diagnosis Discrepancy		☒
Primary Tumor Site Discrepancy		☒
HIPAA Discrepancy		☒
Prior Malignancy History		☒
Duplicate/Synchronous Primary Noted		☒
Case is (check):	☒ QUALIFIED / ☐ DISQUALIFIED	
Reviewer Initials: <i>BB</i>	Date Reviewed: <i>3/26/11</i>	

Specimen Description

Surgical Pathology Report

Patient Name: [REDACTED]
 Accession #: [REDACTED]
 Med. Rec #: [REDACTED]
 Submitting Physician: [REDACTED]

*****Clinical History*****
 History: Multinodular thyroid.

100-0-3

carcinoma, papillary, thyroid 8260/3
 Site: thyroid, NOS 673.9 per 4/3/11

*****Final Pathologic Diagnosis*****

- A. Thyroid gland, total thyroidectomy:
- Papillary thyroid carcinoma of right lobe.
 - See synoptic report.
 - Surgical excision margins are not involved
 - Tumor at less than 1 mm from inked margin of excision.

[REDACTED]
 Electronically Signed By
 [REDACTED]
 [REDACTED]

UID: B06679A8-F5E6-42AD-AE62-EB08C37B5033
 TCGA-FE-A232-01A-PR

Redacted



*****Synoptic Report*****

SYNOPTIC REPORT FOR THYROID CANCER:
 Specimen type: total thyroidectomy
 Tumor location: right lobe of thyroid
 Tumor size: 3.5 x 3.0 x 2.0 cm
 Histologic type: Papillary thyroid carcinoma, with oncocytic features.
 Tumor grade: well differentiated
 Lymphatic/vascular invasion: absent
 Capsular invasion: yes
 Tumor extent:
 Confined to the thyroid: no
 Multifocal: no
 Infiltration of adjacent structures: minimal extension into perithyroid soft tissue.
 Margins: not involved; tumor at less than 1 mm from inked margin of excision.
 Additional pathologic findings:
 - Lymphocytic thyroiditis
 - Nodular hyperplasia of left lobe.
 Special studies:
 Immunohistochemical stains are performed on sections of a

Sex: F

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Capsular invasion: yes

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Multifocal: no

Infiltration of adjacent structures: minimal extension into perithyroid soft tissue.

Margins: not involved; tumor at less than 1 mm from inked margin of excision.

Additional pathologic findings:

- Lymphocytic thyroiditis
- Nodular hyperplasia of left lobe.

Special studies:

Immunohistochemical stains are performed on sections of a paraffin-embedded tissue for calcitonin, thyroglobulin, and TTF-1. The neoplastic cells are positive for TTF-1 and thyroglobulin, and are negative for calcitonin. All immunohistochemical control sections are adequate. The findings are consistent with a papillary thyroid carcinoma with oncocytic features.

pTNM: pT3, pN0, pMX.

The above synoptic report complies, in slightly modified form, with the guidelines of the College of American Pathologists and the Association of Directors of Anatomic and Surgical Pathology for the reporting of cancer specimens

*****SPECIMEN(S) RECEIVED:*****

Thyroid, REG

*****GROSS DESCRIPTION:*****

The specimen is received in one properly labeled container with the patient's name and accession number.

A. The specimen is designated total thyroid and consists of a 32.8 gram thyroid gland that is 9.5 x 5.0 x 3.0 cm. The right lobe is approximately 10% larger than the left lobe. The external surface is tan-gray and wrinkled, with no attached parathyroid glands or lymph nodes. The specimen is inked as follows: right lobe-blue, left lobe-black, isthmus-green. Sectioning of the right lobe reveals a 3.5 x 3.0 x 2.0 cm well-defined, tan-gray, granular, firm nodule within the inferior pole, which focally abuts the capsule. The nodule is approximately 20% necrotic, and does not involve the isthmus. Multiple (under 10) tan, gelatinous, soft, well-circumscribed nodules are noted within the left lobe, which range from 0.3 to 0.5 cm in greatest dimension. The nodules comprise approximately 30% of the left lobe parenchyma. The isthmus is unremarkable. The uninvolved parenchyma is tan-red and rubbery. The entire right lobe nodule is submitted for microscopic examination. (P) 12

Summary of Cassettes: A1-9, entire right lobe node nodule with adjacent capsule and normal thyroid; A10, sections of isthmus; A11-12, sections of left lobe including nodules

Lab Use Only: [REDACTED]

Gross description by: [REDACTED]

[REDACTED]

[REDACTED]

[REDACTED]